When an ASHA hears a warning sign in a village home, the PHC should hear it too— while the visit is still fresh.

ASHAs are women from the communities they serve and often the first point of contact for rural families. The general norm is about one ASHA per 1,000 people, with local exceptions.[1] But visit details can begin as handwritten notes, and digital reporting can depend on typing, a small screen and usable data. ASHA-Vani lets the worker report by speaking into a phone.

THE GAP FROM VILLAGE VISIT TO PHC AWARENESS

A blood-pressure reading and a symptom may be important in the moment, but on paper they can become just another line to enter and review later. The gap between noticing a concern and making it visible to the PHC is the problem ASHA-Vani addresses.

A visit, confirmed aloud

ILLUSTRATIVE · SYNTHETIC PATIENT

ASHA	Rekha Devi ka BP dedh sau by pachhanve hai, lagatar tez sar dard hai, aathva mahina chal raha hai.
ASHA-VANI	BP 150 by 95, aathva mahina, tez sar dard. Sahi hai?
ASHA	Haan, sahi hai.

The current draft screening rule flags this example for PHC review. It can appear as **unverified during the call** and becomes **verified after the ASHA confirms**.

HOW THE CALL BECOMES A RECORD GNANI VOICE IN · CONFIRMED INFORMATION OUT

01	**Listen** GNANI PRISMA	Hindi/Hinglish speech streams from an 8 kHz phone call and becomes a live transcript.
02	**Understand** STRUCTURED EXTRACTION	A language model extracts patient, measurement and symptom fields; “dedh sau by pachhanve” becomes 150/95.
03	**Check and read back** RULES + GNANI TIMBRE	Code checks values and applies readable screening rules. Timbre asks follow-up questions and speaks the captured values.
04	**Confirm and share** PHC REGISTER	The ASHA can correct details. After confirmation, the PHC sees the verified visit and screening flag.

Prisma SPEECH TO TEXT | **Timbre** TEXT TO SPEECH

Built for safety: people make care decisions.

Unclear or implausible values prompt a repeat. Deterministic rules flag patterns; the language model never assigns clinical risk. PHC staff own follow-up and referral decisions. This is decision support, not diagnosis or automated dispatch. Draft rules require clinician review.

DEMO & NEXT PROOF

Demo: browser simulator and Exotel phone-line audio use synthetic patient details; no real patient recordings. **Next:** measure transcription and field-extraction quality, response time, and safe handling of unclear measurements.